

UNDERWRITING

Medical & Travel Insurance

Underwriter Workflow Reference

Underwriter Workflow

Medical insurance underwriting: process, key considerations, and decision flow

DOCUMENT	Underwriter Workflow
SCOPE	Medical Insurance Underwriting
VERSION	1.0
STATUS	For review

1 Medical Insurance Underwriting Workflow

Medical (health) underwriting differs from general property/casualty underwriting because the risk being priced is a person's health, not an asset. This applies both to standalone medical/health insurance and to the medical benefit component of travel insurance policies. The steps below run in sequence for each applicant.

Step 1 — KYC & Compliance Verification

Before risk assessment begins, verify the applicant's identity and eligibility:

- Passport document
- Proof of address
- Sanctions / PEP (politically exposed person) screening

This is a regulatory/AML requirement, not a risk-pricing step, but incomplete or failed KYC blocks underwriting from proceeding and can trigger an automatic decline regardless of medical risk.

Step 2 — Application & Medical Declaration

Applicant completes a health questionnaire disclosing pre-existing conditions, current medications, past surgeries/hospitalizations, and family medical history.

Step 3 — Age & Demographic Risk Screening

Age bands, gender, occupation, and lifestyle factors (smoking, alcohol use, hazardous activities/sports) are assessed, since these strongly correlate with claims frequency and severity.

Step 4 — Pre-Existing Condition Assessment

- Determine whether a condition is disclosed and how it affects eligibility.
- Apply **exclusions** (condition permanently excluded from cover), **waiting periods** (condition covered only after a set time on the policy), or **loadings** (extra premium for elevated risk).
- For travel insurance specifically, pre-existing conditions are a major driver of claim denials, so accurate disclosure and clear policy wording are critical.

Step 5 — Medical Evidence Gathering

For higher sums insured, older applicants, or flagged conditions, request additional evidence: medical exams, doctor's reports (attending physician statements), lab results, or teleunderwriting interviews.

Step 6 — Risk Classification / Rating Tiers

Applicants are grouped into standard, substandard (rated up), or declined categories based on the aggregated medical risk, sometimes using actuarial mortality/morbidity tables or automated underwriting rules engines.

Step 7 — Coverage Limits & Sub-Limits

Set overall sum insured plus medical-specific sub-limits (e.g., emergency medical treatment, hospitalization, evacuation/repatriation, dental emergencies, maternity) appropriate to the risk profile.

*This is the underwriter selecting and adjusting cover for an individual applicant within an already-built product — distinct from **medical cover setup**, the upfront product/actuarial work of designing the plan template and rate tables that underwriters then apply.*

Step 8 — Pricing / Loading Application

Calculate the base premium, then apply any medical loadings, age-banding multipliers, or destination/duration factors (for travel medical cover) to reach the final premium.

Step 9 — Decision, Terms & Policy Issuance

Approve as standard, approve with exclusions/loadings, or decline. Clearly document any medical exclusions or waiting periods on the policy schedule so claims can be assessed consistently later.

Step 10 — Ongoing Claims Feedback Loop

Medical claims experience is fed back into underwriting guidelines and pricing models to keep risk assessment accurate over time (e.g., adjusting loadings for conditions that are producing higher-than-expected claims).

2 Workflow Summary

The complete underwriting sequence, from compliance checks through to the claims feedback loop.



3 Key Considerations

Factors specific to medical and travel medical underwriting that shape disclosure, pricing, and compliance decisions throughout the workflow above.

- **Disclosure accuracy** — Non-disclosure or misrepresentation of medical history is the leading cause of claim disputes.
- **Age thresholds** — Many travel medical policies have hard age cutoffs or require medical screening above a certain age (e.g., 65+, 70+).
- **Destination risk** — Cost of medical treatment varies drastically by country (e.g., US healthcare costs vs. regional averages), affecting pricing and sub-limits.
- **Trip duration** — Longer trips increase exposure to both illness and pre-existing condition flare-ups.

- **Regulatory compliance** — Medical underwriting must comply with local insurance regulations on permissible risk factors and non-discrimination rules.
- **KYC vs. underwriting** — KYC confirms *who* the applicant is (compliance/AML); underwriting assesses *what risk* they present (health, age, destination). They run in parallel at intake, but a KYC failure overrides the underwriting outcome.
- **Family size** — For family/group travel medical plans, the number of dependents affects pricing (per-head vs. flat family rate, child discounts), risk aggregation (each member can trigger an independent claim), and how sub-limits are allocated (shared vs. per-person).